CUSTOMER DISCOVERY ASSIGNMENT

Health Insurance Cashless Claims

Why are authorization requests taking longer than expected?

INDUSTRY Health Insurance **FOCUS** Cashless Authorization

01 CUSTOMER BACKGROUND

A large health insurance company provides medical insurance coverage to individuals, families, and corporate employees. The company works with a network of hospitals that offer cashless treatment facilities to eligible policyholders.

When a policyholder requires hospitalization, the hospital submits a cashless authorization request to the insurance company. The insurance company reviews the request before communicating its decision to the hospital. Multiple applications and operational teams support this process.

02 CUSTOMER PROBLEM STATEMENT

Over the past several months, the insurance company has received increasing complaints from policyholders and network hospitals regarding delays in cashless authorization and claims processing.

Some hospitals report that authorization requests remain pending longer than expected. In certain cases, hospitals need to contact the insurance company repeatedly to understand the status of a request. Policyholders have also raised concerns about the uncertainty they experience while waiting for authorization decisions.

The operations team acknowledges that employees spend considerable time reviewing requests, clarifying information, communicating with hospitals, and resolving pending cases. However, the company does not have a sufficiently clear understanding of why these delays occur or which parts of the process contribute most to them.

Different stakeholders have expressed different opinions about the problem. Management is concerned that continued delays could affect operational efficiency, hospital relationships, customer satisfaction, and the organization's reputation.

03 INITIAL CUSTOMER CONVERSATION

> *"We have invested in digital platforms to support our cashless insurance operations. However, we continue to receive complaints from hospitals and policyholders about delays in processing authorization requests.*
>
> *Our operations teams are working hard, but we still face challenges in processing requests efficiently. Some stakeholders believe the problem is with our technology, while others believe our operational processes need improvement.*
>
> *We do not want to invest in another solution without understanding the actual problem. We want your team to investigate what is happening, identify the underlying causes, and help us determine where improvements would create the greatest business value."*

— Chief Operating Officer, initial discovery meeting

04 YOUR ROLE

You are a Forward Deployed Engineer invited to participate in the initial customer discovery engagement. The underlying causes have not been established; the scale and financial impact are unquantified; the current process and system capabilities have not been fully documented; and no solution has been approved.

Your task: Understand the problem, identify what is unknown, and ask the right questions before recommending any technology or process change.

NINE-BOX CUSTOMER DISCOVERY CANVAS

Use this worksheet to document evidence and questions. Do not fill unknowns with guesses.

Key: F Confirmed fact (stated in the brief / meeting) A Assumption (to be validated) U Unknown (needs an answer) D Decision (made, or pending with an owner)

CUSTOMER / PROJECT: Large health insurer (unnamed) / Cashless authorization delays

FDE / TEAM: Ratheesh

DATE / VERSION: 31 Oct 2026

01 CUSTOMER & STAKEHOLDERS

Who owns the problem? Who uses the process? Who decides?

- F Chief Operating Officer: sponsor in the discovery meeting; wants causes understood before any investment.
- F Operations / claims processing team: reviews requests, clarifies information, resolves pending cases.
- F Network hospitals: submit requests; report pending cases and chase for status.
- F Policyholders: wait on decisions; raised concerns about uncertainty.
- F Management: concerned about efficiency, hospital relationships, satisfaction, reputation.
- A A digital platform / IT team owns the "multiple applications". Not named in the brief.
- U Who owns the problem end to end (ops, IT, medical, network management)? Who holds the budget?
- U Who are the day-to-day users (intake, medical reviewers, call centre)? Is a TPA involved?
- D Pending: COO to name one accountable business owner and the decision maker for any change.

02 BUSINESS OBJECTIVE

Why change? What triggered the request? What matters?

- F Trigger: increasing complaints from hospitals and policyholders over the past several months.
- F Management worries about operational efficiency, hospital relationships, customer satisfaction and reputation.
- F Customer wants the underlying causes found and the highest-value improvement areas identified.
- D Made by COO: no new solution investment until the actual problem is understood.
- A The objective is to cut authorization turnaround time. The brief says "delays" but sets no target.
- U Why now: any regulatory, contractual or competitive trigger?
- U Which concern ranks first: cost, hospital retention, satisfaction, reputation?
- U Scope: authorization only, or also claim settlement / reimbursement?

03 EXPECTED OUTCOMES

What should improve? How will success be measured?

- U No success measure or target has been stated. Candidate measures to agree (not yet decided): authorization turnaround time (request to decision), % pending beyond agreed time, repeat status enquiries per case, complaint volume, hospital / policyholder satisfaction.
- U Is there an existing SLA (internal, hospital contract, regulatory)? "Longer than expected" is measured against what?
- U What improvement would count as success, by when?
- D Pending: COO to agree 2-3 success measures and targets once the baseline exists.

04 CURRENT PROCESS / AS-IS

What happens from request to closure? Where are handoffs?

- **F** Hospital submits a cashless authorization request when a policyholder needs hospitalization.
- **F** Insurer reviews the request, may clarify information with the hospital, then communicates a decision.
- **F** Multiple applications and operational teams support the process.
- **F** Staff spend considerable time reviewing, clarifying, communicating and resolving pending cases.
- **U** Step-by-step flow, handoffs and owners from submission to decision. Not documented yet.
- **U** Submission channel (portal, email, phone, fax)? Request types (planned, emergency, enhancement, discharge)?
- **U** Which steps are manual vs automated? Where do requests wait? How many clarification rounds occur?

05 CORE PROBLEM & PAIN POINTS

What is going wrong? What is verified?

- **F** Requests stay pending longer than expected (reported by hospitals; not yet measured).
- **F** Hospitals contact the insurer repeatedly for status; policyholders face uncertainty.
- **F** Stakeholders disagree on the cause: technology vs process. Neither view is verified.
- **U** Root cause is not established. Which stage contributes most to the delay?
- **A** Hypotheses to test, not conclusions: incomplete or incorrect hospital submissions; manual medical-review queues; disconnected applications and re-keying; poor status visibility; staffing peaks.
- **U** Is delay concentrated in certain hospitals, treatments, request types or times?

06 BUSINESS IMPACT & BASELINE

How often? What cost, time, risk or rework results?

- **F** Complaints are increasing; management sees risk to efficiency, relationships, satisfaction and reputation.
- **U** Volume: requests per month; how many are delayed; current average and 90th-percentile turnaround.
- **U** Cost: staff hours per request, repeat-contact volume, rework.
- **U** Revenue / relationship risk: hospital attrition, policyholder churn.
- **U** Compliance risk: regulatory or contractual turnaround obligations and penalties.
- **U** Complaint count and trend: how is it recorded today?

07 DATA, SYSTEMS & CONSTRAINTS

What systems, data, rules and dependencies exist?

- **F** The company has invested in digital platforms; multiple applications exist.
- **U** Which applications (names, vendors, owners), what each does, how they integrate? Is there a hospital-facing portal?
- **U** What data is captured (timestamps per step, status, reason codes)? Can delay be measured today?
- **U** Business rules: eligibility, required documents, approval limits, auto vs manual adjudication.
- **U** Constraints: budget, timeline, health-data privacy, hospital contracts.
- **A** Part of the process depends on manual work by the operations team; the extent is unknown.

08 REQUIREMENTS & MVP OPTIONS

What capabilities might help? What is the smallest, useful pilot?

- **F** No solution has been approved.
- **D** Deferred: no requirements or MVP until root cause and baseline are known.
- **A** Candidate areas to test later, not requirements: status tracking and visibility, SLA / escalation monitoring, submission completeness checks.
- **U** Smallest useful pilot cannot be defined yet. It depends on which stage causes most delay and whether the cause is process, data or system.

09 VALIDATION & NEXT STEPS

What must be confirmed? Who owns each decision?

- **U** Confirm: process walkthrough and shadowing of the ops team. Owner: Ops lead (A).
- **U** Confirm: 3-6 months of timestamped request data to set the baseline. Owner: IT / data team (A).
- **U** Confirm: interviews with 3-5 hospitals and policyholders. Owner: network / customer service (A).
- **U** Confirm: inventory of applications and integrations. Owner: IT (A).
- **D** Pending: success measures, scope and business owner. Decision owner: COO.